Barbour Orthopaedics & Spine
Exceptional care for an active life

Patient:	**Najmos Sakib**
Date of Birth:	12/20/1998
Date:	09/30/2024 09:00 AM
Visit Location:	**Gwinnett Office**
Provider:	Linville MD, Douglas A

Chief Complaint: Neck pain and Mid back pain

History of Present Illness

Najmos Sakib is a 25-year-old male with neck, mid back sprains/strains following a motor vehicle accident that occurred on 09/09/2024. On the date of injury, the patient reports being the restrained driver of a vehicle traveling approximately 65 MPH when another vehicle traveling approximately 70-75MPH impacted the back of the patient's vehicle at a rapid velocity while driving down the highway. Upon impact, the patient sustained severe forwards and backwards whiplash injuries to the neck and mid-back. Patient's vehicle sustained damages as a result of the impact. He did not seek emergency medical care immediately following his accident. Due to continuation of pain, the patient followed up at a chiropractic office for further evaluation where the patient was started on a weekly adjustment treatment plan. Patient was ultimately referred for MRIs of the neck, mid back due to continued pain, Patient has an appointment at Elite on 09/30/2024. Patient has been using over the counter medications for pain relief. Patient states that he works in UBER and food delivery and has been limited on the ability to work due to his care situation. Patient has not had history of pain, other injury, previous accidents, prior treatment, or work absence related to the neck, mid back, or before this accident. Patient presents today for initial evaluation.

1. Neck pain

Patient complains of constantly aching and tight 6 out of 10 neck pain localized across the base of the neck and upper traps. It is worse with flexion, extension and rotation. Patient denies any radiation of pain, weakness, numbness, or tingling down the bilateral upper extremities at this time.

2. Mid back pain

Patient complains of constantly aching and dull 5-6 out of 10 mid back pain localized across shoulder blades. It is worse with sitting still. Patient denies any radiation of pain, weakness, numbness, or tingling down the bilateral lower extremities at this time.

Past Medical History

Patient reported no relevant past medical history.

Past Surgical History

Patient reported no relevant past surgical history.

Current Medication:

Brand	Generic Name	Dose	Sig Description	Start Date
TIZANIDINE HCL	tizanidine HCl	4 mg	take 1 tablet by oral route every 6 - 8 hours as needed not to exceed 3 doses in 24 hours	09/30/2024
MELOXICAM	meloxicam	15 mg	take 1 tablet by oral route every day	09/30/2024

Allergies:

Ingredient	Reaction (Severity)	Medication Name	Comment
NO KNOWN ALLERGIES			

Review of Systems:

System	Neg/Pos	Details
Constitutional	Negative	Fatigue, Fever and Night sweats.
Respiratory	Negative	Cough and Dyspnea.
Cardio	Negative	Chest pain, Cyanosis and Irregular heartbeat/palpitations.
GI	Negative	Constipation, Diarrhea and Vomiting.
GU	Negative	Dysuria and Hematuria.
Endocrine	Negative	Cold intolerance and Heat intolerance.
Neuro	Positive	Headache.
Neuro	Negative	Difficulty walking and Dizziness.
Psych	Negative	Anxiety and Depression.
Integumentary	Negative	Rash.
Hema/Lymph	Negative	Easy bleeding and Easy bruising.
Allergic/Immuno	Negative	Environmental allergies and Food allergies.

Physical Exams:

Exam	Details
Cervical	General: Normal gait and balance. Loss of lordosis. Range of Motion: Range of motion mildly limited to 75% in all planes secondary to pain and guarding. Palpation: 2+ tenderness to palpation over bilateral cervical paraspinal musculature and trapezii with associated spasming. 2+ tenderness over the midline C4-T1. N/V: Deep tendon reflexes 2+ in bilateral upper extremities. Radial pulses 2+ bilaterally. Sensation to light touch intact and equal over dermatomes C4-T1. Strength: Right shoulder abduction 5/5, left shoulder abduction 5/5; Right wrist extension 5/5, Left wrist extension 5/5; Right wrist flexion 5/5, Left wrist flexion 5/5; Right grip strength 5/5, Left grip strength 5/5; Right finger abduction 5/5, Left finger abduction 5/5. Special Testing: Pain with Spurling's testing. Negative Lhermitte's. Negative Hoffmans. Negative Wartenberg's. Negative Tinel's at the wrists and elbows bilaterally.
Thoracic	Active range of motion is mildly limited with extension. No pain with compression of the rib cage. Respiratory rate, depth, and rhythm is within normal limits. The patient reports no shortness of breath or difficulty breathing. 1+ tenderness with palpation to the right thoracic paraspinal and periscapular musculature with spasm noted. 1+ tenderness over midline thoracic spine.

X-ray Report:

Ordered	Performed	Indication/Dx	Study	Impressions	Findings	Comments
09/30/2024	09/30/2024	M54.2 Cervicalgia Pain	C-Spine 2 view	See findings.	Loss of cervical lordosis. I see no evidence of fractures, dislocations, or any other acute bony abnormalities. Osteophyte noted anterior C6	
09/30/2024	09/30/2024	S23.3XXA Sprain of ligaments of thoracic spine, initial encounter Pain	Thoracic Spine X-ray (2 Views)	See findings.	I see no evidence of fractures, dislocations, or any other acute bony abnormalities.	

Assessment/Plan

#	Detail Type	Description
1.	Assessment	Sprain of ligaments of cervical spine, initial encounter (S13.4XXA).
2.	Assessment	Post-traumatic headache, unspecified, intractable (G44.301).
	Plan Orders	Neurology.
3.	Assessment	Cervicalgia (M54.2).
4.	Assessment	Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA).
	Plan Orders	Therapy Order. Clinical information/comments: Treatment: Chiropractic Treat Date of Injury: 09/09/2024 Physician Goals: Activities of daily living, Education, Increased function, Pain relief Other: Continue chiro modalities per therapist discretion Weekly Frequency: 3 Duration: 4 weeks Number of visits: 12 Therapy Order.

Summary of Visit:

Najmos Sakib is a 25-year-old male with neck, mid back sprains/strains following a motor vehicle accident that occurred on 09/09/2024.

1) Neck: signs and symptoms of a sprain/strain of the cervical spine with concern for disc injury. Proceeding with previously scheduled MRIs to guide interventional treatment.

2) Mid-back: signs and symptoms of a sprain/strain of the thoracic spine with concern for disc injury. Proceeding with previously scheduled MRIs to guide interventional treatment.

I had a long discussion today with the patient regarding their condition and treatment options moving forward. I dispensed Meloxicam 15 mg #30, to take once daily with food for pain and inflammation. I dispensed Zanaflex 4 mg #60 to be taken every 8 hours as needed for muscle spasming. I warned the patient that this medication may make them drowsy and should avoid driving after taking the medication. I recommend patient treat with chiropractic therapy, and provided and demonstrated a formalized HEP program for their injuries, as I believe this will be an important part of their rehabilitation plan going forwards. I recommend the patient continue with his previously scheduled

MRIs at Elite Radiology for his mid-back and neck. Due to persistent headaches since his accident I recommend the patient be evaluated by a neurologist. I am dispensing a TrueStim device in office today. This is a neuromuscular electrical stimulating device for muscle weakness, muscle strengthening and muscle spasms. This device will help with pain, improve stability, and restore proper function. He is currently unable to work due to his vehicle status but is able to return under light duty restrictions. He will return to the clinic for 2 weeks for re-evaluation and to review his MRIs. He states that he understands and agrees with this plan of care.

Referrals

Status	Physician	Reason	Timeframe	Appointment
ordered	Referrals: Neurology. Evaluate and treat			

Rendering Provider: Linville, Douglas A

CC Providers

Electronically signed by Douglas A. Linville MD on 09/30/2024 01:44 PM

3547 Peachtree Industrial Blvd. Suite 4
Duluth, GA 30096
Phone: (770) 299-1332
Fax: (770) 741-0097

Hight-Field MRI & X-Ray

PATIENT NAME:	NAJMOS,SAKIB		
PATIENT ID:	1188128	**REFERRING PHYSICIAN:**	RICARDO HEINIS, DC
DOB:	12/20/1998	**REFERRING PHONE:**	(678) 985-0444
DOS:	10/4/2024 4:00:00 PM	**REFERRING FAX:**	(678) 730-1052

FINAL REPORT

EXAM DESCRIPTION: MRI CERVICAL WITHOUT CONTRAST

INDICATION: The patient complains of neck pain subsequent to motor-vehicle accident on 09/09/2024 with new onset of cervicalgia (M54.2). No history of related surgery is provided.

TECHNIQUE: An MRI of the cervical spine is obtained using sagittal T1 and T2-weighted images as well as a set of para-axial T2-weighted images through the intervertebral discs of the cervical spine without the administration of gadolinium contrast. There are no previous studies or reports of previous studies available for comparison or correlation at the time of the interpretation.

FINDINGS: The cerebellar tonsils have a normal anatomic relationship with the foramen magnum. The vertebral body heights are maintained. The odontoid is intact.

At C2-C3, there is no neural foraminal narrowing or central spinal canal stenosis.

At C3-C4, there is broad-based bulge of the disc without stenosis.

At C4-C5, there is posterior bulge of the disc without stenosis.

At C5-C6, there is posterior bulge of the disc indenting the ventral aspect of the thecal sac without neural foraminal stenosis.

At C6-C7, there is anterior spurring and posterior bulge of the disc indenting the ventral aspect of the thecal sac.

At C7-T1, there is no neural foraminal narrowing or central spinal canal stenosis.

The cervical spinal cord is normal in signal and morphology.

IMPRESSION:

AT C3-C4, THERE IS BROAD-BASED BULGE OF THE DISC WITHOUT STENOSIS.

AT C4-C5, THERE IS POSTERIOR BULGE OF THE DISC WITHOUT STENOSIS.

AT C5-C6, THERE IS POSTERIOR BULGE OF THE DISC INDENTING THE VENTRAL ASPECT OF THE THECAL SAC
WITHOUT NEURAL FORAMINAL STENOSIS.

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Hight-Field MRI & X-Ray

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DOS:	10/4/2024 4:00:00 PM	**REFERRING FAX:**	(678) 730-1052

AT C6-C7, THERE IS ANTERIOR SPURRING AND POSTERIOR BULGE OF THE DISC INDENTING THE VENTRAL ASPECT
OF THE THECAL SAC.

NO ACUTE FRACTURE OF THE CERVICAL SPINE IS IDENTIFIED.

Doug Hornsby MD

doughornsby@msn.com

Electronically signed on: 10/10/2024 11:54
Transcribed by: Velazquez, Grace on: 10/10/2024 09:50

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Hight-Field MRI & X-Ray

PATIENT NAME:	NAJMOS,SAKIB		
PATIENT ID:	1188128	**REFERRING PHYSICIAN:**	RICARDO HEINIS, DC
DOB:	12/20/1998	**REFERRING PHONE:**	(678) 985-0444
DOS:	10/4/2024 4:00:00 PM	**REFERRING FAX:**	(678) 730-1052

FINAL REPORT

EXAM DESCRIPTION: MRI THORACIC SPINE W/O CONTRAST

INDICATION: The patient complains of neck and midback pain subsequent to motor-vehicle accident on 09/09/2024 (M54.6). No history of related surgery is provided.

TECHNIQUE: MRI of the thoracic spine without gadolinium contrast. Magnetic resonance imaging was performed of the thoracic spine utilizing a localizing sagittal cervical pulse sequence. Sagittal T1 and T2-weighted images of the thoracic spine and a set of para-axial T2-weighted images of the thoracic spine were provided for interpretation.

COMPARISON: No previous studies or reports of previous studies were available for comparison or correlation at the time of the interpretation.

FINDINGS: The vertebral body heights are maintained. There is anatomic alignment. The thoracic spinal cord is normal in signal and morphology. The descending thoracic aorta is normal caliber. There is no evidence of pleural effusion. The paravertebral musculature is normal in MRI appearance.

At T12-L1, there is anterior spurring without stenosis.

At T11-T12, there is anterior spurring without stenosis.

The remainder of the intervertebral disc spaces is preserved. The cortical margins are intact. There is no neural foraminal narrowing or central spinal canal stenosis.

IMPRESSION:

AT T12-L1, THERE IS ANTERIOR SPURRING WITHOUT STENOSIS.

AT T11-T12, THERE IS ANTERIOR SPURRING WITHOUT STENOSIS.

NO ACUTE FRACTURE OF THE THORACIC SPINE IS IDENTIFIED.

THE THORACIC SPINAL CORD IS NORMAL IN SIGNAL AND MORPHOLOGY.

Doug Hornsby MD

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Hight-Field MRI & X-Ray

PATIENT NAME:	NAJMOS,SAKIB		
PATIENT ID:	1188128	**REFERRING PHYSICIAN:**	RICARDO HEINIS, DC
DOB:	12/20/1998	**REFERRING PHONE:**	(678) 985-0444
DOS:	10/4/2024 4:00:00 PM	**REFERRING FAX:**	(678) 730-1052

Electronically signed on: 10/10/2024 13:46
Transcribed by: Velazquez, Grace on: 10/10/2024 12:02

Barbour Orthopaedics & Spine
Exceptional care for an active life

Patient: **Najmos Sakib**
Date of Birth: 12/20/1998
Encounter Date: 11/04/2024 11:00 AM

Patient Contact Information:
Address: 4341 Elvie Way , Hoschton, GA, 30548-
Cell Phone: (470)239-5212 **Home Phone:**
Patient Number: 70884

THERAPY ORDERS

Ordered Date	Ordered By	Diagnosis	Primary Sx	Summary	Effective Date	Expiration Date
11/04/2024	Linville MD, Douglas A	M50.21: Other C3-C4 cervical disc displacement M50.221: Other cervical disc displacement at C4-C5 level M50.222: Other cervical disc displacement at C5-C6 level M50.223: Other cervical disc displacement at C6-C7 level		Treatment: Chiropractic Treat Date of Injury: 09/09/2024 Physician Goals: Activities of daily living, Education, Increased function, Pain relief Other: Continue chiro modalities per therapist discretion Duration: Up to Therapist Additional Dx: M53.82: Other specified dorsopathies, cervical region, S23.3XXD: Sprain of ligaments of thoracic spine, subsequent encounter Therapy Order: External Referral		

Electronically signed by Linville MD, Douglas A on 11/04/2024 at11:48 AM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

Initial Exam

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Amanda Ruiz, DC
Date of Service: 9/20/2024

SUBJECTIVE

HISTORY OF PRESENT ILLNESS

He has never experienced this condition before.

PRIMARY COMPLAINT

Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT

Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT

Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT

Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

PAST, FAMILY, SOCIAL HISTORY

Surgeries/Hospitalizations: none
Allergies: none
Medications: Depression ,anxiety, ADHD
Prior conditions: none

Father: High Blood Pressure,High Cholesterol
Mother: LBP

REVIEW OF SYSTEMS

MUSCULOSKELETAL: Joint Pain/Swelling, Stiffness, Muscle pain, Neck pain, Stiff neck, Back pain, Head injury and Birth defects
NEUROLOGICAL: Numbness, Headaches, Difficulty speaking and Concussion
PSYCHIATRIC: Anxiety, Depression, Mood swings, Difficult sleeping, Nervousness and Tension
RESPIRATORY: Cough and Wheezing
SKIN: Itching/Burning, Scars and Discolorations
MEN'S HEALTH ISSUES: Dripping urination
GENERAL: Recent weight gain, Loss of sleep and Fatigue

The following are negative: ALLERGIC-IMMUNOLOGIC, CARDIOVASCULAR, CONSTITUTIONAL, EAR/NOSE/THROAT, ENDOCRINE, EYES, GASTROINTESTINAL, GENITOURINARY and HEMATOLOGY/LYMPH.

Collision occurred on 9/9/2024 at approximately 2:00 AM.
I got rear ended on the highway
Symptoms reported: Headache
Single, work as an uber driver

Mr. Sakib was the driver of the vehicle, a 2015 Toyota Prius, at the time of the collision. There was rear, bumper damage to the vehicle. The collision happened while he was moving at I985 exit 4 in Buford, GA and occurred at night. The weather was clear. Street surface was dry. The impact on the vehicle was at the rear end. Upon impact, the brakes were none, and the car moved over 10 feet. Mr. Sakib was seated in the vehicle as a driver and was wearing a seat belt with the shoulder harness and with the headrest. The vehicle is equipped with air bags and they did not deploy. Mr. Sakib saw the impact coming. Upon impact, his head was looking ahead and he was thrown forward. His body hit seatbelt. There was head trauma. Mr. Sakib remembers the collision happening.

The patient reports to this office due to injuries received in an MVC

OBJECTIVE

VITALS

Height: 163cm in
Temperature: 98.7°F Pulse rate: 88 BPM (relaxed) SpO_2: 99%.
Blood Pressure (seated): right -N/A left-138/75 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8.

Tenderness

Left Side

- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)

Right Side

- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Posterior Thumb (mild-moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)
- Lower Back

Right Side

- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)

Trigger Points

Left Side

- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)

Right Side

- Cervical Spine (mild-moderate)
- Thoracic Spine (mild-moderate)

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Cervical distraction [1]	Positive	Positive	
Foraminal compression test[2]	Positive	Positive	
Maximal foraminal compression test[3]	Positive	Positive	
O'Donogue maneuver[4]	Positive	Positive	
Shoulder depression test[5]	Positive	Positive	
Spinal percussion test-cervical[6]	Positive	Positive	
Schepelmann's sign[7]	Positive	Positive	
Spinal percussion test-thoracic[8]	Positive	Positive	

[1]Relief of radicular pain suggests nerve root compression. Decreased local pain could indicate facet impingement. Increased local pain could indicate strain or sprain.

[2]Radicular pain could indicate pressure on the nerve root. Localized pain could indicate sprain or strain or foraminal encroachment.

[3]Increased peripheral pain could indicate radiculopathy or IVF encroachment. Local neck pain could indicate joint or muscle damage.

[4]Pain during resisted ROM or isometric contraction suggests muscle strain. Pain during passive assisted motion suggests ligamentous strain.

[5]Pain in the arm could indicate adhesions of the nerve roots. Local pain could indicate sprain or strain or adhesions of the shoulder joint capsule.

[6]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

[7]Pain on the concave side may indicate intercostal neuritis. Pain on the convex side may be due to intercostal myofascitis.

[8]Localized pain suggests possible vertebral fracture. Radicular pain indicates possible disc lesion.

REFLEXES

Tricep and Brachioradialis are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	25° (50% of normal)	Pain, Spasm, Guarding	
Extension	40° (66% of normal)	Pain, Spasm, Guarding	
Left lateral flexion	25° (55% of normal)	Pain, Spasm, Guarding	
Right lateral flexion	25° (55% of normal)	Pain, Spasm, Guarding	
Left rotation	40° (50% of normal)	Pain, Spasm, Guarding	
Right rotation	45° (56% of normal)	Pain, Spasm, Guarding	

Wrist ROM - estimated

Test	Measurement	Limited By	Comment
Adduction	L: R: 50° (76% of normal)	Pain, Spasm, Guarding	Pain on the thumb

MUSCLE STRENGTH

Upper Muscle Strength

Location	Left	Right	Comment
Cervical flexors/extensors	4/5	4/5	
Cervical lateral flexon left/right	4/5	4/5	

0-No-muscle-activity; 1-Flicker-of-movement; 2-Movement-with-gravity-eliminated; 3-Movement-against-gravity-only; 4-Movement-against-gravity-with-some-resistance; 5-Movement-against-gravity-with-full-resistance.

DERMATOMES

Posterior

Left-Cervical and Right-Cervical are within normal limits.

PHYSICAL EXAMINATION

Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.

Gross musculoskeletal deformities: WNL.
There are no contraindications to chiropractic care.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
17. Unspecified superficial injury of right thumb, init encntr (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

LONG-TERM GOALS

- Decrease pain to pre-injury status.
- Restore tolerance to normal activities of daily living.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

There is no past medical history that would contribute to the current exam findings. The current complaints are a result of this MVC.

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique was provided specifically to C5; T3; T4; T7, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Diagnostic Tests: X-ray cervical 4-5 views (72050) was provided due to significant traumatic mechanism of injury.
- Diagnostic Tests: X-ray thoracic 2 views (72070) was provided due to significant traumatic mechanism of injury.
- New Patient evaluation and management service, 99203, was provided today.

The patient presents today for a New Patient Exam. The course of care and treatment plan was discussed with the patient. The patient had an opportunity to ask questions and a treatment plan was given to the patient which will be 3 times per week for 4 weeks.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 09/20/2024 05:55 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 9/23/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Reviewed the X-rays with the patient.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Posterior Thumb (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Worked on the patients thumb with some Activator and some manual therapy.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
17. Unspecified superficial injury of right thumb, init encntr (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation

- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.

- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique was provided specifically to C2; **C3**; T3; **T5**; T6; T7, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- **Extraspinal CMT (98943) consisting of activator technique was provided to Right Finger, to reduce pain reported at initial encounter by 25%.**

Review x-rays with the patient. The patient had an opportunity to ask questions and understand the findings. Due to positive findings on those X-rays, the patient will be referred for an MRI of the Cervical and Lumbar spine for further assessment.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 09/23/2024 11:25 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 9/30/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

- Posterior Thumb (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, init (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, init (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, init (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unsp motor-vehicle accident, traffic, init (V89.2XXA)
17. Unspecified superficial injury of right thumb, init encntr (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

COMPLICATING FACTORS

Contributing Conditions

- Auto Accident

Aggravating Conditions

- No contraindications to chiropractic manipulation
- ADL's

PLAN

CARE PLAN DETAILS

Based on the findings, there will be 3 stages of care; Acute, Rehabilitative, and Corrective.

Based on the patient's condition, re-evaluations are planned, for each stage of care, to assess the benefits of care and ensure functional improvement.

During the **Acute stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 50%. This will be provided 3 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 12 visits.

During the **Rehabilitative stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Back and Neck, to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 75%. This will be provided 2 times per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 8 visits.

During the **Corrective stage**, the following services are expected to be provided:

- 98941 - CMT 3-4 Regions consisting of diversified technique and Instrument adjusting will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97014 - Electrical Stimulation consisting of interferential will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 97012 - Mechanical Traction consisting of intersegmental traction will be performed to the Neck and Back, to reduce pain reported at initial encounter by 100%. This will be

provided 1 time per week for 4 weeks.

- 97010 - Ice/Heat consisting of hot pack will be provided to reduce pain reported at initial encounter by 100%. This will be provided 1 time per week for 4 weeks.
- 99213 Re-evaluations will be performed once every 4 visits.

The patient will be re-evaluated at the end of care, with Est. pt. level 3, evaluation and management (99213), at which time the next phase of care will be discussed.

Care Plan and treatment are subject to change dependent on patient's compliance and response to care.

TODAY'S TREATMENT

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique was provided specifically to C2; C3; **C4**; T5; T6; T7, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 09/30/2024 01:34 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Amanda Ruiz, DC
Date of Service: 10/2/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Posterior Thumb (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, **Thompson technique** was provided specifically to **C5**; **T1**; **T2**; T5; T6, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 50%.
- **Extraspinal CMT (98943) consisting of Instrument adjusting, diversified technique, to reduce pain reported at initial encounter by 25%.**

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 10/02/2024 12:48 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 10/4/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient expressed no change since last visit.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Posterior Thumb (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

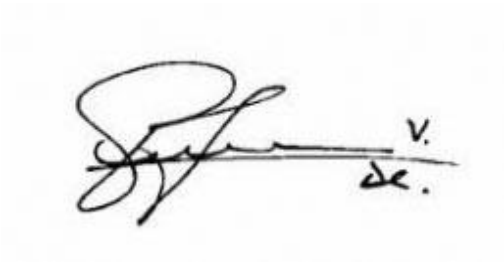
Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C3**; **C5**; T1; T2; T5; T6, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to **Neck**, **Back**, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/04/2024 02:14 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 10/7/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient **expressing that he is doing well and with only minor discomforts**.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Posterior Thumb (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

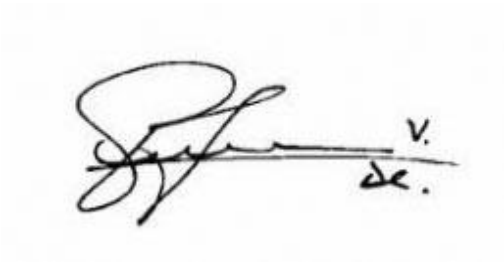
Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C3; **C4**; T1; T2; **T7**; **T8**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/07/2024 01:51 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Amanda Ruiz, DC
Date of Service: 10/9/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Posterior Thumb (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- **CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C2; T1; T3; T4; Pelvis, to reduce pain reported at initial encounter by 50%.**
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 10/09/2024 12:34 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 10/16/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient was with some upper back stiffness.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Posterior Thumb (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

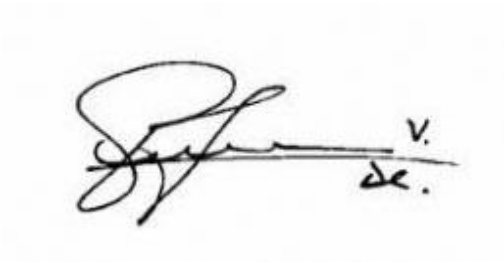
Based on subjective and objective data, today's treatment consists of:

- **CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C2; C7; T1; T3; T4; T7; T8, to reduce pain reported at initial encounter by 50%.**
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/16/2024 01:47 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 10/18/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

- Posterior Thumb (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C1**; **C2**; C7; T3; T4; T7; T8; **T9**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/18/2024 01:17 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Amanda Ruiz, DC
Date of Service: 10/21/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Posterior Thumb (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C5**; **C6**; **T5**; T9; **T10**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 10/21/2024 12:38 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 10/23/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

SECOND COMPLAINT
Neck

- Aching pain: 6/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 6/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 7/10
  Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient stated that he is progressing well with his adjustments.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)

- Thoracic Spine (Mild-Moderate)
- Posterior Thumb (Mild-Moderate)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)
- Lower Back

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Trigger Points

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild-Moderate)

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Guarded - The patient's condition is not expected to improve in the near future.

PLAN

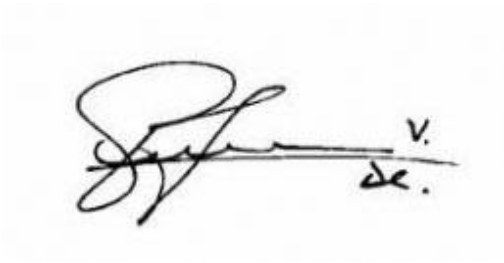
Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C4**; **C5**; T5; **T8**; **T9**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 10/23/2024 02:45 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Rita Patel, DC
Date of Service: 10/25/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: **From 6 to 1/10 (50% decrease)**
  Described as **occasional (up to 25% of day)** of **minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL)**; increased by ADL's; decreased by **Chiropractic care**.

SECOND COMPLAINT
Neck

- Aching pain: **From 6 to 5/10 (10% decrease)**
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: **From 6 to 4/10 (20% decrease)**
  Described as **occasional to intermittent (up to between 25% and 50% of day)** of **slight to moderate intensity**; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: **From 7 to 1/10 (60% decrease)**
  Described as **infrequent** of **minimal intensity (requires very little or no modification to ADL)**; increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

VITALS
Height: 163cm in Weight: 160 lbs
Temperature: 96.5°F Pulse rate: 89 BPM (relaxed) SpO_2: 98%.
Blood Pressure (seated): right -N/A left-122/71 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**

Tenderness

Left Side

- Thoracic Spine **(decreased 5-10% to mild)**
- Cervical Spine (Mild-Moderate)

Right Side

- Thoracic Spine **(decreased 5-10% to mild)**
- Cervical Spine (Mild-Moderate)
- Posterior Thumb **(decreased 10-20% to minimal)**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 10-20% to minimal)**
- Lower Back

Right Side

- Cervical Spine **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 10-20% to minimal)**

Trigger Points

Left Side

- Cervical Spine **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 10-20% to minimal)**

Right Side

- Cervical Spine **(decreased 5-10% to mild)**
- Thoracic Spine **(decreased 10-20% to minimal)**

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Test	Left	Right	Comment
Cervical distraction [1]	Positive	**WNL**	
Maximal foraminal compression test[2]	Positive	Positive	

[1]Relief of radicular pain suggests nerve root compression. Decreased local pain could indicate facet impingement. Increased local pain could indicate strain or sprain.

[2]Increased peripheral pain could indicate radiculopathy or IVF encroachment. Local neck pain could indicate joint or muscle damage.

Foraminal compression test, O'Donogue maneuver, Shoulder depression test, Spinal percussion test-cervical, Schepelmann's sign and Spinal percussion test-thoracic are within normal limits.

REFLEXES

Tricep and Brachioradialis are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Flexion	**45° (90% of normal,**	Pain	

	increased by 80%)		
Extension	**50° (83% of normal, increased by 25%)**	Pain, Guarding	
Left lateral flexion	**40° (88% of normal, increased by 60%)**	Pain, Spasm, Guarding	
Right lateral flexion	**40° (88% of normal, increased by 60%)**	Pain, Guarding	
Left rotation	**75° (93% of normal, increased by 87%)**		
Right rotation	**75° (93% of normal, increased by 66%)**		

Wrist ROM - estimated

Test	Measurement	Limited By	Comment
Adduction	L: R: **WNL**		Pain on the thumb

MUSCLE STRENGTH

Upper Muscle Strength

Location	Left	Right	Comment
Cervical flexors/extensors	4/5	4/5	
Cervical lateral flexon left/right	4/5	4/5	

0-No-muscle-activity; 1-Flicker-of-movement; 2-Movement-with-gravity-eliminated; 3-Movement-against-gravity-only; 4-Movement-against-gravity-with-some-resistance; 5-Movement-against-gravity-with-full-resistance.

DERMATOMES

Posterior

Left-Cervical and Right-Cervical are within normal limits.

PHYSICAL EXAMINATION

Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

ASSESSMENT

Stage of Care: Acute

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Prognosis has changed from **Guarded to Fair** - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 0% to 80% of goal achieved.**
- Restore tolerance to normal activities of daily living: **Progressed from 0% to 80% of goal achieved.**

Patient is **showing improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **slightly-decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C4; C5; **T1**; **T2**; **T5**; T8; T9, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.
- **Established Patient evaluation and management service, 99213, was provided today.**

Reexam today. the new care plan will be 2 times per week for 4 weeks.

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 10/25/2024 01:20 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Amanda Ruiz, DC
Date of Service: 11/1/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild)

- Thoracic Spine (Mild)
- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C3**; **C4**; C5; T1; T2; **T4**; **T7**; T8, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 11/01/2024 12:29 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 11/4/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient had been holding his adjustments in the past few visits and has not suffered from any acute pain.

OBJECTIVE

PALPATORY FINDINGS

Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**

Tenderness

Left Side	Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild)

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild)
- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C3; C4; T1; T2; T7; T8, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.

- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/04/2024 02:01 PM

Iceberg Health Morrow
1544 Southlake Pkwy
Unit 9F
Morrow, GA 30260-0287
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Amanda Ruiz, DC
Date of Service: 11/8/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild)

- Thoracic Spine (Mild)
- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C2**; **C3**; **C5**; T1; T2; **T6**; **T7**; T8, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Amanda Ruiz, DC
Date: 11/08/2024 12:21 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 11/11/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

Reviewed MRI with the patient.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild)

- Thoracic Spine (Mild)
- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C2; C3; **C4**; T2; **T3**; T6; T7; T8, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.
- **Established Patient evaluation and management service, 99212, was provided today.**

MRI Results were reviewed today with the patient. The patient had the opportunity to ask questions and understand the findings. Due to positive findings, the patient will be referred to a specialist for further assessment.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/11/2024 12:53 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 11/15/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient was with **no change since his last visit**.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild)

- Thoracic Spine (Mild)
- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

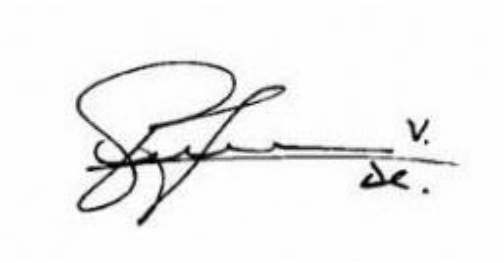
Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C2; C3; C4; T2; T3; T6; T7; T8, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 11/15/2024 02:48 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 12/2/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient was **expressing** no change in his **pain levels compared to** last visit.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild)

- Thoracic Spine (Mild)
- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

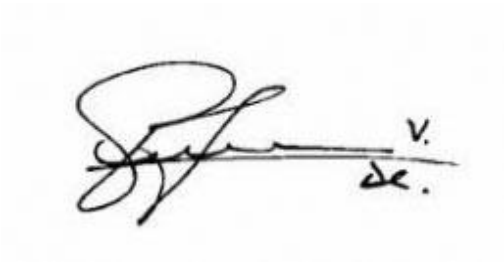
Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C3; C4; T3; **T4**; T7; T8; **T9**, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 12/02/2024 02:52 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 12/4/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild)

- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Patient was experiencing muscle tightness on the right.

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

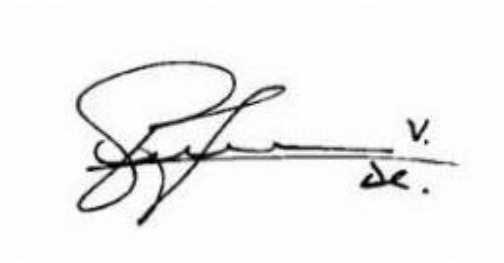
Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to **C4**; **C5**; T3; T4; T7; T8, to reduce pain reported at initial encounter by 50%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 12/04/2024 02:53 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 12/6/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient stated that he is feeling good and has no discomfort at the present moment.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild)

- Thoracic Spine (Mild)
- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

ASSESSMENT

Stage of Care: Rehabilitative

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

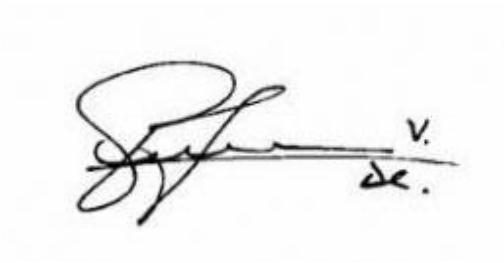
Based on subjective and objective data, today's treatment consists of:

- **CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique, Thompson technique was provided specifically to C3; C4; T3; T4; T9; Pelvis, to reduce pain reported at initial encounter by 50%.**
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 50%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to , to reduce pain reported at initial encounter by 50%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 50%.

Patient received care without incident.



(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 12/06/2024 12:43 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 12/11/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8.
Tenderness

Left Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild)

Right Side

- Cervical Spine (Mild-Moderate)
- Thoracic Spine (Mild)

- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side
- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side
- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Patient seems to be holding his adjustments well, there is little to no changes since his last visit.

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 3-4 Regions (98941) consisting of Instrument adjusting, diversified technique was provided specifically to C3; **T4**; **T5**; T9; **Right L5**, to **reduce pain reported at initial encounter by 100%**.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to **reduce pain reported at initial encounter by 100%**.

- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to **Neck**, **Back**, to **reduce pain reported at initial encounter by 100%**.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to **reduce pain reported at initial encounter by 100%**.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 12/11/2024 02:35 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 12/16/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient is feeling well today, no specific pain or discomfort.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild)

- Thoracic Spine (Mild)
- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- **CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique was provided specifically to C2; C3; C5; T3; T4; T5; T6; T7, to reduce pain reported at initial encounter by 100%.**
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 12/16/2024 02:26 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 12/23/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient **did not have any changes** to **report**.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild)

- Thoracic Spine (Mild)
- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique was provided specifically to C2; C3; T4; T5; T6; T7, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 12/26/2024 08:40 AM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Daily Note

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Ricardo Heinis, DC
Date of Service: 12/27/2024

SUBJECTIVE

PRIMARY COMPLAINT
Head

- Aching pain: 1/10
  Described as occasional (up to 25% of day) of minimal to slight intensity (symptom is not alleviated by any amount of modification to ADL); increased by ADL's; decreased by Chiropractic care.

SECOND COMPLAINT
Neck

- Aching pain: 5/10
  Radiates to (posterior) left shoulder and right shoulder. Described as frequent (up to 75% of day) of moderate intensity (requires significant or complete modification to ADL); increased by ADL's; decreased by rest.

THIRD COMPLAINT
Upper back, Mid back

- Aching pain: 4/10
  Described as occasional to intermittent (up to between 25% and 50% of day) of slight to moderate intensity; increased by ADL's; decreased by rest.

FOURTH COMPLAINT
Right thumb

- Pain: 1/10
  Described as infrequent of minimal intensity (requires very little or no modification to ADL); increased by changing positions and ADL's; decreased by rest.

Patient **expressed some tightness and soreness in his upper thoracic**.

OBJECTIVE

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**

Tenderness

Left Side

- Cervical Spine (Mild-Moderate)

Right Side

- Cervical Spine (Mild-Moderate)

- Thoracic Spine (Mild)

- Thoracic Spine (Mild)
- Posterior Thumb (Minimal)

Spasm/Hypertonicity

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)
- Lower Back

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Trigger Points

Left Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

Right Side

- Cervical Spine (Mild)
- Thoracic Spine (Minimal)

ASSESSMENT

Stage of Care: Corrective

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
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9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Fair - The patient can expect to have a reduction of their symptom and may require ongoing rehabilitation.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique was provided specifically to C2; C3; **T2**; **T3**; **T4**; T5, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided

specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.

- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.

Patient received care without incident.

(Electronic Signature)
Provider Name: Ricardo Heinis, DC
Date: 12/27/2024 12:42 PM

Iceberg Health Lawrenceville
1670 McKendree Church Road Suite 400B
Lawrenceville, GA 30043-4100
(678) 505-0000

*Changes since previous Note have been **formatted differently.***

Exam

Patient Name: Najmos Sakib
Date of Birth: 12/20/1998

Provider: Rita Patel, DC
Date of Service: 1/2/2025

SUBJECTIVE

PRIMARY COMPLAINT
Head

SECOND COMPLAINT
Neck

- **Dull** pain: **From 5 to 1/10 (40% decrease)**
  Described as **infrequent** of **minimal intensity (requires very little or no modification to ADL)**; increased by ADL's; decreased by **Chiropractic care** and rest.

THIRD COMPLAINT
Upper back, Mid back

FOURTH COMPLAINT
Right thumb
Patient **states he sometimes feels neck pain**.

OBJECTIVE

VITALS
Height: 163cm in Weight: 160 lbs
Temperature: 98.8°F Pulse rate: 87 BPM (relaxed) SpO_2: 99%.
Blood Pressure (seated): right -N/A left-145/91 mmHg
Demeanor: coherent and relaxed.

PALPATORY FINDINGS
Spinal asymmetry/fixation: C1; C2; C3; C4; C5; C6; C7; T1; T2; T3; T4; T5; T6; T7; T8**.**

Tenderness

Left Side

- Cervical Spine **(decreased 10-20% to minimal)**
- Thoracic Spine **(decreased 10-20%)**

Right Side

- Cervical Spine **(decreased 10-20% to minimal)**
- Thoracic Spine **(decreased 10-20%)**
- Posterior Thumb **(decreased 5-10%)**

Spasm/Hypertonicity

Left Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10%)**

Right Side

- Cervical Spine **(decreased 5-10% to minimal)**
- Thoracic Spine **(decreased 5-10%)**

- Lower Back

Trigger Points

Left Side

- Cervical Spine **(decreased 10-20%)**
- Thoracic Spine **(decreased 5-10%)**

Right Side

- Cervical Spine **(decreased 10-20%)**
- Thoracic Spine **(decreased 5-10%)**

GAIT

Ambulation: Normal.
Gait: Normal.

ORTHOPEDIC AND NEUROLOGICAL TESTS

Cervical distraction , Foraminal compression test, **Maximal foraminal compression test**, O'Donogue maneuver, Shoulder depression test, Spinal percussion test-cervical, Schepelmann's sign and Spinal percussion test-thoracic are within normal limits.

REFLEXES

Tricep and Brachioradialis are within normal limits bilaterally.

RANGE OF MOTION

Cervical ROM - estimated

Test	Measurement	Limited By	Comment
Right lateral flexion	40° (88% of normal)	Pain, Guarding	

Flexion, **Extension**, **Left lateral flexion**, **Left rotation** and **Right rotation** are within normal limits.

Wrist ROM - estimated

Test	Measurement	Limited By	Comment
Adduction	L: R: WNL		Pain on the thumb

MUSCLE STRENGTH

Upper Muscle Strength

Cervical flexors/extensors and **Cervical lateral flexon left/right** are within normal limits bilaterally.

DERMATOMES

Posterior

Left-Cervical and Right-Cervical are within normal limits.

PHYSICAL EXAMINATION

Neck [supple, thyroroid non-palpable, no enlarged cervical lymph nodes palpable]: WNL.
General [well nourished, no acute distress]: WNL.
Skin [no skin sensitivity reported, free of lesions, rashes, eczema, upper/lower extremities]: WNL.
Head [eyes – symmetric appearance, conjunctiva white, no discharge, erythema, lid lag or swelling]: WNL.
Ears [no discharge or deformities noted to external ears]: WNL.
Nose [no sinus tenderness, epistaxis, or discharge]: WNL.
Oropharynx [lips free of lesions, cyanosis, no halitosis]: WNL.
Extremities – [upper/lower extremities free of edema, cyanosis, varicosities, cellulitis, pedal pulses 2+]: WNL.
Gross musculoskeletal deformities: WNL.

ASSESSMENT

DIAGNOSES

1. Tension-type headache, unspecified, not intractable (G44.209)
2. Sprain of ligaments of cervical spine, initial encounter (S13.4XXA)
3. Sprain of ligaments of thoracic spine, initial encounter (S23.3XXA)
4. Sprain of ligaments of lumbar spine, initial encounter (S33.5XXA)
5. Strain of muscle, fascia and tendon at neck level, initial encounter (S16.1XXA)
6. Strain of muscle and tendon of back wall of thorax, initial encounter (S29.012A)
7. Strain of muscle, fascia and tendon of lower back, initial encounter (S39.012A)
8. Segmental and somatic dysfunction of cervical region (M99.01)
9. Segmental and somatic dysfunction of thoracic region (M99.02)
10. Segmental and somatic dysfunction of lumbar region (M99.03)
11. Cervicalgia (M54.2)
12. Pain in thoracic spine (M54.6)
13. Low back pain, unspecified (M54.50)
14. Muscle spasm of back (M62.830)
15. Sleep deprivation (Z72.820)
16. Person injured in unspecified motor-vehicle accident, traffic, initial encounter (V89.2XXA)
17. Unspecified superficial injury of right thumb, initial encounter (S60.931A)

PROGNOSIS

Prognosis has changed from **Fair to Good** - Symptomatic and functional recovery is expected in approximately 8-12 weeks.

LONG-TERM GOALS

- Decrease pain to pre-injury status: **Progressed from 80% to 90% of goal achieved.**

Patient is **showing improvement** since the prior visit, He is **progressing as anticipated**. Intensity is **decreased** since the prior visit.

PLAN

Based on subjective and objective data, today's treatment consists of:

- CMT 1-2 Regions (98940) consisting of Instrument adjusting, diversified technique was provided specifically to C2; **C5**; T2; T5, to reduce pain reported at initial encounter by 100%.
- Electrical Stimulation (97014) consisting of interferential was provided specifically to , to reduce pain reported at initial encounter by 100%.
- Mechanical Traction (97012) consisting of intersegmental traction was provided specifically to Neck, Back, to reduce pain reported at initial encounter by 100%.
- Ice/Heat (97010) consisting of hot pack was provided specifically to , to reduce pain reported at initial encounter by 100%.
- **Established Patient evaluation and management service, 99214, was provided today.**

Final exam today. The patient has reached maximum medical improvement in this office. The patient will continue with home care instructions if symptoms persist.

Patient received care without incident.

(Electronic Signature)
Provider Name: Rita Patel, DC
Date: 01/02/2025 12:16 PM

Addenda

The patient is being discharged today after a course of care with residual deficits in function from the injuries sustained from their Motor Vehicle Accident.
According to the American Medical Association (AMA) Guides to the Evaluation of Permanent Impairment 5th edition this patient may have a Permanent whole-body impairment of up to 25%.
In order to prevent further damage this patient will have to receive regular treatment at the rate of 2 times per month for life.
Approximate $88,740 in conservative lifetime treatment costs.

Lifetime Treatment Costs = [Avg. Life expectancy - Current Age] X 12 X $150= Lifetime treatment.

The average Male Life Expectancy is 75.3 years.
The average Female Life Expectancy is 81.1 years.

Electronic Signature: Patel, Rita DC

Added on Jan 02, 2025